

Telehealth, GLP-1s, Peptides, and Regulatory Risk

A healthcare-investing workbook for separating growth stories from legal, medical, and regulatory dependencies.

Audience	Market students learning healthcare, telehealth, and regulatory-risk analysis.
Core skill	Build a regulatory dependency map before judging a healthcare company thesis.
Current as of	April 21, 2026

Teaching principle

Healthcare investing must start with humility. A drug can be real, demand can be huge, and the stock can still be risky if the business model depends on regulatory exceptions, advertising claims, prescribing rules, reimbursement, or compounding policy.

Learning objectives

- Explain GLP-1s, peptides, telehealth, compounding, and FDA-approved drugs in plain language.
- Separate medical demand from business-model durability.
- Understand why FDA, DEA/HHS, litigation, and company filings matter to a telehealth thesis.
- Create a regulatory dependency map for Hims and Hers or any healthcare platform.
- Avoid giving medical advice while still analyzing healthcare companies.

1. Conversation anchor

In the April 21, 2026 session, the Hims and Hers discussion turned into a broader lesson about GLP-1s, peptides, telehealth, compounded drugs, and government regulation. The key market idea was that a company can have strong customer demand but still depend on whether regulators allow a product, channel, advertisement, or prescribing model.

The student should learn to ask: 'Which part of this growth story is medical demand, and which part is regulatory permission?'

Boundary for students

This tutorial is business and regulatory education. It is not medical advice. Students should not recommend drugs, doses, compounding sources, or treatment plans. Clinical questions belong with licensed clinicians.

2. Plain-English definitions

Term	Plain-English meaning	Why investors care
Peptide	A short chain of amino acids. Some medicines are peptides or peptide-like molecules.	The label can attract wellness hype; legal status and evidence vary by product.
GLP-1	A hormone pathway used by drugs such as semaglutide and tirzepatide to affect blood sugar, appetite, and weight.	Demand is large, but supply, pricing, patents, and FDA rules shape profits.
FDA-approved drug	A drug reviewed by FDA for safety, effectiveness, labeling, and manufacturing quality.	Approved products have clearer legal footing than unapproved compounded copies.
Compounded drug	A custom-prepared medicine made by a pharmacy or outsourcing facility for a specific need.	Can be lawful in specific circumstances but cannot simply bypass FDA approval rules.
503A / 503B	Sections of U.S. law governing traditional pharmacy compounding and outsourcing facilities.	The route affects manufacturing, marketing, and shortage-based exceptions.
Telehealth	Healthcare delivered remotely using digital tools.	Scales distribution, but prescribing and advertising rules remain critical.

3. Demand is not the same as durable revenue

GLP-1 demand is real, but a business model built around GLP-1 access can still change quickly. The ADA's 2026 Standards of Care press release highlighted new guidance on obesity medications and dose individualization for people with diabetes [S3]. That tells students the medical category is important. It does not tell them which company will win economically.

For investors, the sharper question is: which revenue is durable under normal rules, and which revenue exists because of a shortage, temporary flexibility, loose advertising, or gray-market behavior?

Layer	Student question	Possible risk
Medical need	Is there strong patient demand and clinical relevance?	Demand may be real but not captured by the company.
Product legality	Is the product FDA-approved, compounded, or unapproved?	Regulators can limit marketing or production.

Layer	Student question	Possible risk
Prescribing channel	Can providers prescribe through telehealth under current rules?	DEA/HHS or state rules can tighten.
Supply chain	Who makes the product and under which standard?	Quality, sourcing, shortage status, and compliance risk.
Marketing	Are claims truthful and not misleading?	Warning letters, lawsuits, or forced ad changes.
Unit economics	Does the platform retain customers profitably?	High acquisition costs or lower margin after switching to branded drugs.

4. Hims and Hers case study

Hims and Hers is useful as a teaching case because it combines consumer demand, subscription healthcare, telehealth distribution, pharmacy/fulfillment, marketing, and GLP-1 regulatory exposure. The company reported approximately \$2.35 billion of revenue for 2025, up 59% year over year, and more than 2.5 million subscribers, up 13% year over year [S7]. That gives students growth evidence.

But the same thesis must include regulatory evidence. In its 2025 Form 10-K, Hims and Hers said FDA's resolution of the semaglutide shortage limits its ability to use 503B outsourcing facilities to provide access to compounded semaglutide on the platform [S2]. In March 2026, the company announced a strategic shift toward a broader assortment of FDA-approved GLP-1s and said it would no longer advertise compounded GLP-1 offerings on its platform or in marketing [S6].

Evidence	Bullish use	Risk use
Revenue and subscribers	Platform demand and cross-selling potential.	Growth may slow if a key product category changes.
FDA-approved GLP-1 access	Cleaner legal footing and partner relationships.	Lower margin or less pricing flexibility than compounded offerings.
Compounded GLP-1 limitation	May still serve clinically necessary exceptions.	Cannot be the same broad growth engine if shortage exceptions end.
Direct-to-consumer brand	Customer acquisition and recurring engagement.	Advertising claims can draw FDA/FTC/legal scrutiny.

5. Regulatory timeline students should know

A timeline prevents students from treating regulation as background noise. In this case, the dates are part of the thesis.

Date	Event	Why it matters to a thesis
Nov. 4, 2024	DOJ announced a Cerebral settlement over business practices tied to controlled-substance prescribing [S8].	Shows telehealth scale can create enforcement risk when incentives affect prescribing.
Feb. 21, 2025	FDA determined the semaglutide injection shortage was resolved [S1].	Reduced the shortage-based rationale for broad compounded semaglutide access.
Jan. 2, 2026	HHS and DEA extended telemedicine flexibilities for prescribing controlled medications through 2026 [S4].	Temporary flexibility can support access but does not remove legitimate-purpose and compliance requirements.
Mar. 3, 2026	FDA warned 30 telehealth companies about false or misleading claims for compounded GLP-1s [S5].	Marketing claims became an explicit enforcement focus.
Mar. 9, 2026	Hims and Hers announced a U.S. weight-loss strategy shift toward FDA-approved GLP-1s [S6].	Company adapted its model after regulatory and market changes.
Jul. 23-24, 2026	FDA Pharmacy Compounding Advisory Committee scheduled discussion of several peptide bulk substances [S9].	Peptide rules could create or close future product categories.

6. The regulatory dependency map

Every healthcare platform has dependencies. Students should map them before building a thesis. A dependency is any rule, partner, license, clinical standard, supply chain, or claim that the business needs in order to keep operating as expected.

Dependency	Question	Evidence to collect
Product status	Is the product approved, compounded, shortage-based, or experimental?	FDA pages, labels, shortage database, company filings.
Clinical need	Does the product serve a medically supported use?	ADA guidelines, peer-reviewed clinical summaries, clinician standards.
Prescriber model	Are providers licensed and prescribing for legitimate medical purposes?	Company disclosures, HHS/DEA rules, state boards.
Marketing claims	Do ads imply equivalence or benefits without support?	FDA warning letters, FTC actions, company ad language.
Supply/fulfillment	Who makes, ships, and monitors quality?	503A/503B status, pharmacy partners, quality disclosures.
Partner risk	Does the model depend on a drugmaker, payer, pharmacy, or lab?	Contracts, litigation, supplier concentration.
Customer economics	Will customers stay if product mix or price changes?	Subscriber growth, churn, gross margin, cohort data.

7. Scenario analysis

Students should not write one heroic forecast. They should write scenarios. The point is not to predict perfectly; the point is to know what would change the thesis.

Scenario	What happens	Likely thesis impact	What to monitor
Approved GLP-1 access expands	More branded options, more dosage/form-factor flexibility, better supply.	Positive for compliant access; margin may depend on partner terms.	Partner announcements, pricing, gross margin.
Compounded GLP-1 enforcement increases	More warning letters, seizures, injunctions, or ad restrictions.	Negative for companies relying on broad compounded-copy marketing.	FDA press releases and company risk updates.
Peptide compounding becomes clearer	FDA/PCAC process creates a more defined path for some substances.	Could open new product categories, but evidence and safety remain key.	PCAC meetings, final rules, company product claims.
Telehealth controlled-substance rules tighten	Special registration or stricter prescribing requirements.	Affects ADHD, psychiatric, and other controlled-substance services more than ordinary wellness products.	DEA/HHS rules, state enforcement.
Customer acquisition cost rises	More competitors bid for the same weight-loss customer.	Growth can continue while profitability weakens.	Marketing expense, churn, CAC payback.

8. How to teach the medical/business split

Students can discuss the business of GLP-1s without giving medical advice. The safe teaching frame is: 'We are analyzing demand, legal channels, revenue durability, and risk disclosures. We are not advising anyone to take, stop, switch, compound, or dose a medication.'

Do teach	Do not teach
How FDA-approved drugs differ from compounded products.	Which GLP-1 or peptide a person should take.
How shortage status can change business models.	How to source compounded medications.
How telehealth prescribing rules affect scale.	How to bypass an in-person medical exam or prescribing rule.
How to read company risk factors and regulatory updates.	Medical dosing, contraindications, or personalized treatment advice.

9. Workshop assignment

Assignment: Regulatory risk card

Create a one-page regulatory risk card for Hims and Hers or another telehealth company. The goal is to identify which parts of revenue are durable, which depend on policy, and which could change suddenly.

Required fields

- Company and product category.
- Is the key product FDA-approved, compounded, or something else?

- Which agency or regulator matters most: FDA, DEA, HHS, FTC, state boards, payers, or courts?
- What event would make the thesis stronger?
- What event would make the thesis weaker?
- What source will you check every month?
- Final classification: investment candidate, trade only, watchlist, or avoid.

Rubric

Category	Excellent	Needs work
Medical/business boundary	Clearly avoids treatment advice and focuses on business evidence.	Mixes stock thesis with personal medical recommendations.
Regulatory map	Identifies specific agencies, rules, dates, and dependencies.	Says 'regulation risk' without details.
Primary evidence	Uses FDA, SEC filings, company releases, HHS/DEA, ADA.	Relies only on social posts or price action.
Scenario thinking	Includes bull, bear, and neutral pathways.	Only argues one outcome.

10. Quick quiz

Ask students to answer before checking notes.

Question	Expected answer
Why can GLP-1 demand be real while a telehealth GLP-1 stock remains risky?	Because demand is only one layer; product status, compounding rules, marketing claims, partner economics, and margins can change.
What changed when FDA resolved the semaglutide shortage?	The broad shortage-based route for certain compounded semaglutide access became more limited.
Why is the Cerebral case relevant even if the product differs?	It shows that telehealth scale plus prescribing incentives can attract enforcement when patient safety or controlled substances are involved.
What is the first source to check for company-specific risk?	The company's SEC filings, especially risk factors and MD&A.;

11. Source pack

ID	Source	How students should use it
T	April 21, 2026 conversation transcript	Conversation context: Hims and Hers, GLP-1s, peptides, telehealth, and regulatory risk.
S1	FDA - GLP-1 compounding policy and semaglutide shortage resolution	Use for official GLP-1 compounding/shortage status.
S2	Hims and Hers 2025 Form 10-K	Use for risk factors and company-specific regulatory exposure.
S3	ADA - Standards of Care in Diabetes 2026 release	Use for medical guideline context, not stock recommendations.
S4	HHS and DEA - telemedicine flexibilities extended through 2026	Use for telehealth prescribing-rule context.

ID	Source	How students should use it
S5	FDA - Warning letters to telehealth companies for compounded GLP-1 marketing	Use for advertising and enforcement risk.
S6	Hims and Hers - Strategic shift for U.S. weight loss business	Use for management's current product-channel strategy.
S7	Hims and Hers - FY 2025 results	Use for revenue, subscriber, and guidance context.
S8	DOJ - Cerebral telehealth settlement	Use as a cautionary case on prescribing incentives and enforcement.
S9	FDA - July 23-24, 2026 PCAC peptide meeting	Use for pending peptide-compounding regulatory developments.

Educational note: This tutorial is for research training only. It is not investment advice, medical advice, or legal advice.